From Strategy to Actions: Prioritizing NCD Prevention and Control

Regional workshop to advance NCD prevention and control in the WHO South-East Asia Region

28–30 October 2025

Jaipur, India

Meeting Report

World Health Organization

South-East Asia Region

SEA-NCD-117

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Cataloguing-in-Publication (CIP) data. CIP data are available at http://apps.who.int/iris.

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Background and objectives of the workshop

Background

Noncommunicable diseases (NCDs), principally cardiovascular diseases (CVDs), diabetes, cancer, and chronic respiratory diseases, impose a major and growing burden on health and development in the South-East (SE) Asia region. These major NCDs share the key risk factors of tobacco use, physical inactivity, alcohol use, unhealthy diet, and air pollution. In 2021, 55% of all deaths in the SE Asia region were due to NCDs, accounting for 9.5 million persons[1]. Of particular concern is the fact that 50% of deaths from NCDs in the region were premature, in the age category of 30–69 years. CVDs and cancer contribute to the highest proportions of premature deaths among deaths due to NCDs in the SE Asia region with the proportions of premature deaths for the conditions being as high as 49% and 66%, respectively[1].

At its 76th session in 2023, the Regional Committee adopted the Resolution on '*SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the South-East Asia Region (SEA/RC76/R5)*'[2]. The resolution calls for sustained and accelerated efforts at both policy and programmatic levels to address key CVD risk factors (tobacco use, high salt intake, and industrial trans-fats) while expanding hypertension and diabetes care through primary health care (PHC). The resolution also specifies a set of four milestones agreed upon by the countries to be reached by 2025. As of December 2024, progress toward the SEAHEARTS milestones suggests that the region is on track to achieve three out of four of its 2025 targets.

Considering the substantial progress made in three of the four SEAHEARTS milestones, and the ongoing momentum in implementing the technical packages, the Member States expressed strong commitment and agreed upon a set of five ambitious yet achievable SEAHEARTS Milestones 2030. Progress of the SEAHEARTS 2025 milestones was presented to the 78th session of the WHO SE Asia Regional Committee in October 2025 and with unanimous agreement from the Member States the Committee decided to extend the timeline of the four milestones of the SEAHEARTS Initiative for another 5 years, i.e. till 2030. Considering the significant burden of novel nicotine products, smokeless tobacco and areca nut related products, WHO in consultation with Member States developed and presented the regional strategic framework for combating smokeless tobacco, novel nicotine products, and areca nut use in the WHO South-East Asia Region (2025–2030) (The Framework) at the 78th Regional Committee. The Framework was endorsed by the Regional Committee vide Resolution SEAR/RC78/R5 for implementation at the country level.

Cancer control in the Region remains a key priority for WHO. The *WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management (2024–2030)*[3], endorsed by the 77th Regional Committee, serves as a roadmap for Member States to

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[1] *Global Health Estimates 2021: World Health Organization; 2024 https://www.who.int/data/gho*

[2] *SEAHEARTS: ACCELERATING PREVENTION AND CONTROL OF CARDIOVASCULAR DISEASES IN THE SOUTH-EAST ASIA REGION*

[3] *WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management 2024–2030.*

strengthen and accelerate national cancer control efforts and defines parameters for monitoring the progress.

Against this background, Regional workshop (hybrid) was organized to target senior government officials/policy makers and programme managers for CVD, NCD risk factors control, cancer control along with clinicians, academicians, researchers, and professional associations from SEAR Member States, to achieve the following objectives:

General objective

To collectively design a strategic approach to advance NCD prevention and control in the WHO South-East Asia Region.

Specific objectives

- To review country-level progress in managing hypertension and diabetes, as well as in controlling tobacco use and unhealthy diets, and to facilitate discussion and consensus on the renewed and extended SEAHEARTS milestones for 2030.
- To introduce and update participants on existing and newly developed WHO tools and guidance for the prevention and control of cardiovascular diseases (CVDs), cancers, and associated NCD risk factors.
- To assist countries in developing national roadmaps that outline priority actions for translating WHO technical packages into effective measures to control CVDs and achieve the SEAHEARTS 2030 milestones.
- To assist countries in implementing the Regional Strategic Framework for combating smokeless tobacco, novel nicotine products, and areca nut use in the WHO South-East Asia Region (2025–2030).
- To design a regional framework for monitoring progress toward the SEAHEARTS 2030 milestones.
- To review progress on national cancer control activities and provide guidance in identifying and prioritizing actions for comprehensive cancer prevention and management.

Summary of the proceedings

Opening session

The Regional workshop on advancing NCD prevention and control in the WHO South-East Asia Region: From Strategy to Action – Prioritizing NCD prevention and control commenced with an inaugural session that set the tone for collective regional action towards achieving the SEAHEARTS 2030 targets (Annex 1- Programme). The session brought together senior officials/policy makers from Ministries of Health, technical staff from WHO country and regional offices, experts, and key partner organizations (Annex 2).

Ms Payden, WHO Representative to India, delivered the Welcome Address, warmly greeting participants and emphasizing the importance of accelerating the implementation of NCD prevention and control strategies. She highlighted India's commitment to advancing public health priorities through strong partnerships and data-driven approaches. Her remarks emphasized that with political commitment, evidence-based interventions, and robust primary healthcare systems, it is possible to achieve tangible progress in reducing the NCD burden across the Region.

Dr Suman Rijal, Director, Department of Healthier Populations and Noncommunicable Diseases (HPN), WHO Regional Office for South-East Asia (SEARO), delivered the opening remarks on behalf of the Officer-in-Charge (OIC), WHO Regional Office for South-East Asia. In delivering the OIC's message, Dr Rijal highlighted the Region's progress since the adoption of the Resolution SEA/RC76/R5 on SEAHEARTS: Accelerating Prevention and Control of CVDs in the South-East Asia Region by Member States in 2023.

He noted that, as of June 2025, **three of the four milestones had shown significant advancement**, with many countries nearing their national targets of placing 100 million people with hypertension and/or diabetes under protocol-based management. Key achievements included the expansion of opportunistic screening across health facilities and workplaces, adoption of simplified clinical management protocols with task shifting to non-physician providers, improved availability of essential medicines through morbidity-based forecasting, and the introduction of digital tracking systems with unique identifiers to monitor control rates. He also pointed out areas requiring intensified attention, notably, the **integration of statins for CVD prevention, validated BP devices, and HbA1C testing**. Dr Rijal reaffirmed WHO SEARO's commitment to supporting Member States through the HEARTS and PEN technical packages to ensure comprehensive NCD care at all levels of the health system. The full message from the Officer in Charge, as delivered by Dr Suman Rijal is attached as Annex 3.

Dr Nalika Gunawardena, Regional Adviser (NCD), WHO SEARO, presented the SEAHEARTS 2025 achievements and lessons learnt. It was emphasized that SEAHEARTS represents the Region's first targeted and measurable collective approach to address NCDs through shared accountability and regional solidarity.

She reported substantial progress toward the four milestones as of June 2025, noting that three milestones have already demonstrated significant achievements:

- **94 million** people with hypertension and diabetes have been enrolled in protocol-based management, **on the path to reach the target of 100 million to be reached by end 2025**
- **2.08 billion** people are covered by at least one WHO SHAKE technical package measure, **surpassing the target of 1 billion to be reached by end 2025**
- **2.03 billion** people are protected from the harmful effects of trans fats through best-practice or complementary policy measures, **exceeding the target of 2 billion to be reached by end 2025**.

Regarding the progress of the national commitments by the Member States towards the collective target of placing 100 million people with hypertension are/or diabetes under protocol-based management, it was noted that several Member States have already reached the commitment while others are on track.

She reported that the extension of SEAHEARTS milestones to 2030 was approved by the 78th Session of the WHO Regional Committee for South-East Asia (October 2025, Colombo), reiterating the region's long-term commitment to lowering the burden of cardiovascular disease. Following were presented as the extended and expanded SEAHEARTS 2030 milestones agreed upon by the Member States.

Dr Alarcos Cieza, Unit Head, Management of Noncommunicable Diseases, WHO Headquarters, delivered a powerful keynote address on "Global Commitments on NCDs: Turning the Tide Together." She provided a global perspective on the evolving commitments made through successive UN High-Level Meetings on NCDs and Mental Health, highlighting over 20 key commitments covering major NCDs, mental health, and shared risk factors. The two targets, 150 million fewer tobacco users and 150 million more people with hypertension controlled to be reached by 2030 were highlighted as being in alignment with the SEAHEARTS 2030 Milestones.

Dr Cieza drew attention to persistent challenges—slowing progress in reducing premature NCD mortality since 2015, rising metabolic and environmental risks, and gaps in medicines, diagnostics, and health workforce capacity. She outlined forthcoming WHO guidance, including the Operational Framework on Access to Essential Medicines and Health Technologies for NCDs, and updates to the Global Breast Cancer Initiative, Cervical Cancer Elimination Initiative, and Global Initiative for Childhood Cancer. She called for stronger integration of NCD and cancer services into primary healthcare to ensure continuity of care across the life course. She also highlighted WHO's expanding work in NCDs in emergencies, noting ongoing collaboration with countries in the region to strengthen preparedness and

continuity of services during humanitarian settings. Dr Cieza further mentioned that additional resources will be made available to the SE Asia region to support these efforts.

Representing the host country, Ms Aradhana Patnaik, Additional Secretary and Mission Director (NHM), Ministry of Health and Family Welfare, Government of India, delivered the Keynote Address. Ms Patnaik highlighted India's major achievements under the National Programme for Prevention and Control of Non-Communicable Diseases (NP-NCD), including large-scale population-based screening for hypertension, diabetes, and oral, breast, and cervical cancers at Ayushman Arogya Mandirs and primary health facilities, covering more than 70% of the eligible 30+ population to date, with a goal of reaching 100% coverage. She emphasized India's strategic push to institutionalize NCD screening through dedicated weekly NCD days, expanded training for Community Health Officers and ASHAs, and planned revisions of the Community-based Assessment Checklist (CBAC) tool to improve risk assessment and follow-up. She further noted India's efforts to broaden the national NCD agenda beyond the initial five conditions, with new initiatives on STEMI, stroke, and COPD, including the rollout of hub-and-spoke models for cardiac and stroke care, and the nationwide expansion of daycare cancer centres in district hospitals. Ms Patnaik highlighted the importance of shifting from screening alone to ensuring continuity of care—timely treatment, affordability, accessibility, and long-term disease control. Reflecting on India's progress and the near achievement of the "75 by 25" target, she urged countries to set more ambitious goals for the next five-year cycle, aligning national strategies with the SEAHEARTS framework. She urged continued regional collaboration to scale up successful models, enhance surveillance systems, and ensure equitable access to NCD services.

The inaugural session concluded with a vote of thanks by Dr Jagdish Kaur, Regional Adviser, Tobacco Free Initiative, WHO SEARO.

Technical session: Extended and expanded SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control

Session	Speakers/Facilitators/Moderators
Regional SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control	Dr Pradeep Joshi, WHO SEARO
A panel discussion on overcoming common challenges to reach the SEAHEARTS 2030 milestones	
Avoiding stock-outs of medicines and technologies through realistic forecasting	Mr Bashier Enoos, WHO HQ *(virtual)* Dr Saroj Kumar, Ministry of Health and Family Welfare, India
Approaches to promote use of statins in primary health care level	Dr Jeyaraj Durai Pandian, WHO CC for Stroke Care, Christian Medical College, Ludhiana, India Dr S. C. Wickramasinghe, Ministry of Health and Mass Media, Sri Lanka
Improving treatment adherence for hypertension through single-pill combinations	Dr Kouamivi Agboyibor, WHO HQ *(virtual)* Ms Tshewang Lhaden, Department of Public Health, Bhutan
Enhancing access to HbA1c to monitor glycaemic control at primary health care level	Dr Aqsha Azhary Nur, WHO HQ *(virtual)* Dr Mateus Ornai Romiso, Ministry of Health, Timor-Leste
Ensuring availability of validated BP measuring devices in primary care level	Dr Swagata Kumar Sahoo, Resolve to Save Lives Dr Gampo Dorji, WHO Country Office Nepal

Session objectives:

- Consensus on 2030 Milestones: Agreement by Member States on the extended and expanded SEAHEARTS 2030 targets for hypertension and diabetes treatment coverage and control.
- Refined country commitments: Initial guidance for Member States to adapt regional milestones into national targets, considering country-specific burden, care cascade gaps, and health system capacity.
- Shared lessons and enablers: Collective review of progress, enablers, and persistent challenges from the 2025 milestones to inform programmatic scale-up.
- Priority actions for scale-up: Identification of key policy and programmatic accelerators (e.g., scale-up of HEARTS technical package, private sector integration, medicines/diagnostics access, digital monitoring).
- Reinforcement of regional solidarity and political commitment to accelerate CVD prevention and control, aligned with SDG 3.4 and global NCD coverage targets.

Day 1 of the Regional workshop focused on extending and expanding the SEAHEARTS milestones to 2030 for improving hypertension and diabetes treatment coverage and control

in the WHO South-East Asia Region. The session opened with a presentation by Dr Pradeep Joshi, Technical Officer (NCDs), WHO SEARO, who outlined extended and expanded milestones for 2030 agreed by countries. The current status of hypertension and diabetes care cascades and the major programme-level gaps in implementing the WHO HEARTS and WHO PEN technical packages were highlighted. Dr Joshi also presented a set of proposed contribution by each Member State towards the SEAHEARTS 2030 Milestones on hypertension and diabetes coverage and control, building on commitments made during the Third Regional Consultation on SEAHEARTS: Extended and Expanded Milestones 2030 (https://iris.who.int/server/api/core/bitstreams/d7722117-79bf-45da-9108-e3ff68aaab33/content). The importance of prioritizing control status in next five years while improving the treatment coverage was emphasized. The fact that improving control status of those on treatment being a critical indicator of overall service quality and decisive factor in preventing premature mortality was noted. Proposed country-level actions required for the next five years to meet the 2030 milestones were discussed in detail.

The session included a panel discussion on overcoming common challenges faced by the Member States to scale up the hypertension and diabetes coverage and control in primary health care settings along with sharing of experiences by representatives of Member States in overcoming them. The panel discussion included topics on:

- Avoiding stock-outs of medicines and technologies through realistic forecasting
- Approaches to promote use of statins in primary health care level
- Improving treatment adherence for hypertension through single-pill combinations
- Enhancing access to HbA1c to monitor glycaemic control at primary health care level
- Ensuring availability of validated BP measuring devices in primary care level.

Mr Bashier Enoos (Technical Officer, WHO Headquarters) discussed strategies to avoid stock-outs of essential NCD medicines and technologies through realistic forecasting, drawing lessons from pilot implementation of WHO's forecasting tool. Dr Saroj Kumar, Director, Noncommunicable Diseases Division, Ministry of Health and Family Welfare, India presented the country's approach to strengthening NCD drug forecasting. He explained that India's federal structure combines a strong centrally developed policy framework with decentralized state-level implementation, supported by digital systems. The NCD portal tracks patient load, while the DVDMS platform monitors medicine availability up to subcenter level. Forecasting is based on morbidity data, which states required to maintain at least a three-month buffer stock. DVDMS enables early identification of potential shortages and redistribution of medicines between facilities to ensure uninterrupted supply. Periodic field verification complements digital monitoring and improves the accuracy of forecasting and stock management.

Dr Jeyaraj Durai Pandian, (Director, WHO Collaborating Centre for Stroke Care at Christian Medical College, Ludhiana), discussed approaches to promote the use of statins at the primary health care level and shared global evidence showing that statins can reduce heart attacks and strokes by nearly one-third and lower mortality by around 15%. He emphasized that despite clear WHO recommendations, statin use in the WHO South-East Asia Region

remains very low (5.5% for primary prevention and 21.9% for secondary prevention[4]). He drew attention to the target of the Global Monitoring Framework on NCDs that at least 50% of eligible people with existing CVD or at high risk will receive effective drug therapies including statins by 2030. He presented that, despite all Member States in the region have statins included in the National List of Essential Medicines and statins are included in the management protocols for primary health care, use of statins at primary care level remains limited in the countries of the region. Dr S. C. Wickramasinghe, Deputy Director General-NCDs, Ministry of Health and Mass Media from Sri Lanka presented how the country has integrated statin use into national treatment protocols and is now practiced in 17 of the country's 25 districts, aided by a digital tool (Simple App) that prompts when patients meet statin eligibility criteria. This has enabled the monitoring of prescriptions through facility indicators and achieve statin use among 70% of patients who require it at primary care level.

Dr Kouamivi Agboyibor (Medical officer- Cardiovascular diseases, WHO HQ) presented on improving treatment adherence for hypertension through single-pill combinations (SPCs), highlighting evidence of multicentre randomized trials that use of SPCs result in 25–30% higher adherence and more consistent blood pressure control with fewer side effects. Ms Tshewang Lhaden (Program Officer, Noncommunicable Diseases Division, Department of Public Health) from Bhutan shared country experiences in poor medication adherence in spite of good service coverage for hypertension and diabetes. The plans of the country in piloting an intervention on giving people an option to access SPCs to gather local evidence on its effect on improving medication adherence was highlighted.

[4]*Use of statins for the prevention of cardiovascular disease in 41 low-income and middle-income countries: a cross-sectional study of nationally representative, individual-level data. Marcus, Maja E et al. The Lancet Global Health, Volume 10, Issue 3, e369 - e379. https://www.thelancet.com/action/showPdf?pii=S2214-109X%2821%2900551-9*

Dr Aqsha Azhary Nur (Consultant- Global Diabetes Compact, WHO HQ) spoke on improving access to point-of-care (POC) HbA1c testing for glycaemic monitoring in primary care. He noted that WHO's 2020 PEN and HEARTS-D guidelines recommend HbA1c as the preferred test for monitoring. High costs and limited availability of machines also constrain access in many LMICs, where plasma glucose is often used for diagnosis. To address these gaps, WHO has developed technical specifications and expanded its prequalification programme to include HbA1c POC devices. He highlighted the WHO-D Card initiative in Ghana, where HbA1c POC testing is included in national guidelines, reimbursed under the health insurance scheme, and being scaled to primary care facilities. Similar adoption is seen in Malaysia. Dr Mateus Ornai Romiso (Head of Noncommunicable Diseases Department, Ministry of Health) from Timor-Leste provided insights on introducing HbA1c testing at municipal levels as well as efforts to scale-up HbA1c analysers to the national essential diagnostics list as well as advocacy efforts to secure finances.

Swagata Kumar Sahoo (Senior Technical Advisor, Resolve to Save Lives) addressed the importance of ensuring availability of validated blood pressure measuring devices in primary care settings to ensure accurate diagnosis and monitoring. He mentioned that 75-80% of automated BPMDs marketed globally are not clinically validated for accuracy. He also presented the WHO recommendation to use validated automated BPMDs that meet international accuracy standards (ISO 81060-2:2018) in primary care settings. He also presented the online registries such as STRIDE BP, MEDAVAL, and Validate BP as the sources to check on the status of validity of BPMDs. Dr Gampo Dorji (Senior Health Policy Advisor, WHO Country Office) from Nepal described the Kavre initiative to strengthen the hypertension care cascade, highlighting how the introduction of clinically validated automated BP devices has improved ease of use and acceptance among health workers. He emphasized the need for continued advocacy to promote use of validated BPMDs across Nepal, noting that although WHO and the Ministry of Health have procured and supplied validated devices in selected districts, a large proportion of health facilities still rely on non-validated aneroid or the analog BP machine. The importance of building a validation culture in the procurement process was underscored, raising awareness among health workers, and ensuring that both facility-level and patient self-monitoring devices meet clinical validation standards.

The session concluded with an engaging question-and-answer segment, where participants discussed solutions for strengthening NCD service delivery in resource-constrained settings. Discussions highlighted the importance of realistic forecasting to avoid stock-outs of essential medicines and technologies, approaches to promote use of statins at the primary health care, and strategies to improve treatment adherence for hypertension through the use of single pill combinations. The need to enhance access to Hb1Ac testing for monitoring glycaemic control and ensuring the availability of validated blood pressure measuring devices across facilities was emphasized upon.

Participants of each country engaged in group work to frame country-specific targets and design national roadmaps for achieving the SEAHEARTS 2030 milestones. Country teams revisited their rapid situation assessment to identify the existing gaps at programmatic and implementation level to accelerate and scale up the implementation of WHO HEARTS

package to reach SEAHEARTS 2030 milestones for hypertension and diabetes coverage and control.

The gaps identified served as the basis for the design of the national roadmap to reach SEAHEARTS milestones 2030, translating the identified gaps into concrete actions with specified timelines, necessary financial, human, and material resources and the responsible technical units. The national roadmaps were formulated under the thematic areas of target setting, early detection, access to medicines and technologies, standard treatment protocols, team-based care, information systems, and accountability mechanisms.

The session concluded with country presentations on national commitments and proposed roadmaps (summarized in Annex 5). The exchanges facilitated peer learning and highlighted innovative approaches to scale up hypertension and diabetes coverage and control in the primary health care settings across the Region.

Countries presented their national roadmaps for hypertension and diabetes treatment coverage and control, outlining strategic actions to expand screening, improve continuity of care, strengthen health information systems, and enhance the quality of NCD service delivery within primary healthcare. The roadmaps reflected country-level ownership and readiness to operationalize SEAHEARTS targets through multisectoral coordination, sustainable financing, and community engagement (Annex 5).

Technical session: Extended and expanded SEAHEARTS milestones 2030: Tobacco control

Session	Speakers/Facilitators/Moderators
Regional strategic framework for combating smokeless tobacco, novel nicotine products and areca nut in the WHO South-East Asia Region (2025-2030)	Dr Jagdish Kaur, WHO SEARO
Country presentations- identification of priority interventions to reach and monitor the SEAHEARTS 2030 on tobacco control	Bangladesh – Dr Asma Sultana Tuli Bhutan – Dr Bhupinder Aulakh, WR DPR Korea – (virtual) India – Dr Avinash Sunthlia, SMO Maldives – Dr Abdul Azeez Hameed, SMO Myanmar – Dr Aye Moe Moe Lwin, NPO Nepal – Dr Lonim Dixit, NPO Sri Lanka – Dr Ananda Rathnayake, Chairman, NATA Thailand – Mr Chirawat, Yoosabai, Legal Officer Timor-Leste – Mr Mario Serekay, Sen. Officer
Updated status of WHO MPOWER implementation – gaps and challenges	Dr Hebe Gouda, *(virtual)* Project Officer, Tobacco Free Initiative, WHO HQ
Tobacco cessation programme in Rajasthan	Dr Zakaria Chauhan, WHO State Programme Officer (Tobacco Cessation), State of Rajasthan, WCO India
Tobacco products testing – updated evidence	Dr Priyamvada Sharma, Incharge, National Tobacco Testing Laboratory, National Institute of Mental Health and Neuro Sciences, Bangalore (NIMHANS), Bangalore, India
Policies for prevention and control of smokeless tobacco	Prof. Shalini Singh, Director, NICPR, Dept. of Health Research, MOHFW India & WHO FCTC Knowledge Hub on Smokeless Tobacco, India

Session objectives

- To provide updated knowledge on the current status of MPOWER package implementation in the WHO South-East Asia Region, based on the Global Tobacco Control Report 2025 (GTCR X), including identified gaps and challenges.
- To make participants identify and prioritize interventions required to achieve the tobacco control objectives of SEAHEARTS 2030, informed by both country presentations and group discussions thereof.
- Introduced countries to the Regional Strategic Framework for Combating Smokeless Tobacco, Novel Nicotine Products and Areca Nut (2025–2030) and get their feedback on the ways to operationalize this framework within their national and subnational contexts.

- To highlight best practices in tobacco cessation services, tobacco product regulation, and smokeless tobacco control from subnational, national, regional, and global experiences, with opportunities to adapt lessons learned to country-specific needs.
- Raise awareness of technical tools, policy options, and guidance documents that can be leveraged to accelerate progress toward SEAHEARTS 2030 tobacco control milestones.

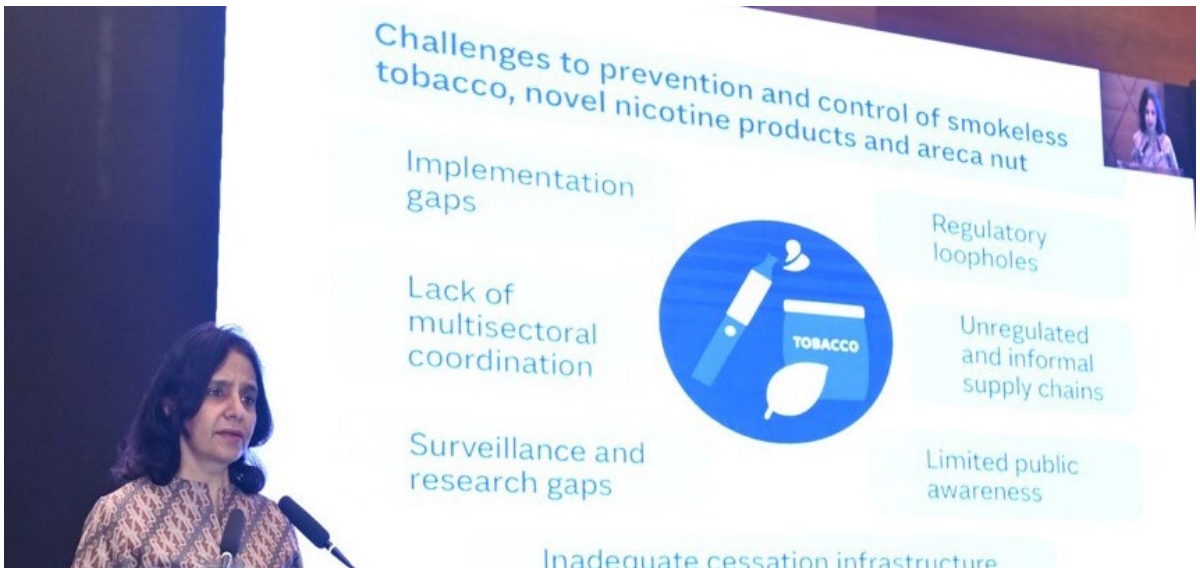
The second day of the Regional workshop focused on tobacco control, a core component of the regional strategy to reduce premature mortality from noncommunicable diseases (NCDs). Sessions explored the latest developments in tobacco and areca nut control frameworks, implementation gaps in MPOWER measures, and country-level innovations to strengthen cessation and regulatory enforcement.

Dr Jagdish Kaur, Regional Adviser (TFI), WHO SEARO, presented an overview of the Regional Strategic Framework for combating smokeless tobacco, novel nicotine products and areca nut in the WHO South-East Asia Region. It was highlighted that the South-East Asia Region faces a uniquely complex tobacco and nicotine landscape dominated by various forms of smoking and smokeless tobacco (SLT), novel nicotine products (NNPs) such as e-cigarettes and nicotine pouches, and areca nut use—often culturally entrenched and unregulated. While several countries in the Region have enacted laws restricting smokeless tobacco, evidence shows that access to these products remain widespread.

The Framework envisions a tobacco-, nicotine and areca nut-free Region and outlines seven strategic objectives focusing on:

- Strengthening national leadership and governance;
- Updating legal and regulatory frameworks;
- Enhancing enforcement and compliance mechanisms;
- Increasing public awareness and communication;
- Expanding cessation services;
- Advancing research, surveillance, and monitoring; and
- Developing national frameworks/mechanisms to address areca nut use.

The Framework aligns with WHO Framework Convention on Tobacco Control (FCTC), MPOWER package, and WHO's "Best Buys," allowing flexibility for country-specific adaptation. Participants emphasized the importance of integrating SLT and NNP control into existing tobacco legislation, ensuring inter-sectoral coordination, and incorporating areca nut control into national public health policies.



Dr Hebe Gouda (WHO-TFI HQ, virtual) presented the updated status of WHO MPOWER implementation- gaps and challenges. Only four countries globally have achieved best-practice levels across all six MPOWER components, with progress slowing in recent years. In the South-East Asia Region, most Member States are on a decreasing trend in tobacco use and are among the fastest globally to reduce prevalence. However, the large number of tobacco users and emerging use of e-cigarettes and novel tobacco products remain major concerns.

Dr Gouda emphasized the need for continuous monitoring, vigilance against industry interference, and innovation to support cessation and youth protection efforts.

The country presentations collectively highlighted strong regional commitment and ongoing efforts to advance tobacco control in line with the SEAHEARTS 2030 targets. Representatives from Member States shared national priorities, emphasizing the need to

strengthen enforcement of smokeless tobacco bans, integrate cessation services into primary healthcare, implement taxation reforms, and enhance cross-sectoral surveillance systems. Countries reported progress in updating legislative frameworks, including amendments to tobacco control laws, enforcing comprehensive bans on advertising and promotion, and fiscal measures to make tobacco products less affordable. Many are expanding cessation support through quitlines, inclusion of nicotine replacement therapy in essential medicine lists, and community-based interventions. Public awareness campaigns and youth-focused initiatives are being scaled up to address the growing use of smokeless tobacco, novel nicotine products, and areca nut. Despite this progress, common challenges persist—such as weak enforcement, industry interference, limited resources, and inadequate data systems. Countries highlighted the importance of multisectoral coordination, capacity building, and continued technical assistance from WHO to accelerate progress and ensure effective monitoring of SEAHEARTS 2030 milestones.

The Tobacco cessation programme in Rajasthan was presented, demonstrating effective integration of tobacco cessation into primary health systems through Ayushman Arogya Mandirs. Over 50 Tobacco Cessation Centres (TCCs) are operational in district hospitals, and around 8,000 Community Health Officers (CHOs) are being trained in brief cessation counselling for tobacco cessation. A poster on brief counselling for Tobacco Cessation was launched on the World Health Day. Convergence with the National Oral Health Programme has expanded reach through dental clinics, with Nicotine Replacement Therapy included in the essential drug list. The initiative exemplifies how decentralized, data-driven action can support large-scale cessation and integration with TB and NCD services.

Dr Priyamvada Sharma (NIMHANS, virtual) presented recent findings from India's tobacco product testing labs network, including the new National Tobacco Testing Laboratory (NTTL) established at NIMHANS. The labs are part of global WHO TobLabNet, serving the

South-East Asia Region. Chemical analyses of various tobacco products reveal significant variation in nicotine content, moisture, and flavor additives across different brands, especially in SLT products, with high levels of bioavailable nicotine and widespread use of menthol and eucalyptol—heightening addiction potential. The testing of tobacco products is envisaged under Articles 9 and 10 of the WHO FCTC regarding product regulation and disclosure, supporting implementation of evidence-based tobacco control policies.

Dr Shalini Singh (WHO FCTC Knowledge Hub on Smokeless Tobacco, ICMR-NICPR) reviewed policies for prevention and control of smokeless tobacco, highlighting India's experience in regulating SLT and pan masala under the various existing laws and policies such as the Food Safety and Standards Act and its Regulations, Plastic Waste Management and Handling Rules, Cigarettes and Other Tobacco Products Act (COTPA) and its amendments, Juvenile Justice (Care & Protection of Children) Act etc. The Knowledge Hub supports SE Asia Region Member States through capacity building, policy advocacy, and regulatory gap analysis, with special attention to surrogate advertising, spit-free public places, and emerging NNPs. Dr Singh emphasized the need to integrate SLT control with broader health, fiscal and environmental policies.

The discussions highlighted that the Region requires comprehensive, multisectoral approaches addressing smokeless tobacco (SLT), novel nicotine products (NNPs), and areca nut in tandem with traditional tobacco control measures. Participants emphasized the need to integrate cessation services into primary healthcare and digital health platforms to effectively reach high-risk populations, while expanding product testing and regulatory frameworks to include SLT and emerging nicotine products. Knowledge exchange and capacity building through regional mechanisms such as the WHO FCTC Knowledge Hub and the global networks such as tobacco testing labs under WHO TobLabNet were identified as essential for inter-country coordination and partnership. It was further highlighted that monitoring of SEAHEARTS 2030 milestones should align closely with MPOWER indicators and SDG target

3.a. to ensure systematic tracking of regional impact. The session concluded with an open discussion reaffirming the collective commitment of Member States and partners to accelerate regional action on tobacco control.

In the area of tobacco control, participants identified several priority actions for accelerated implementation of evidence-based policies and tools at the country level, including sustained, evidence-based mass media campaigns, scaling up of accessible and high-quality cessation services, and increasing tobacco taxation on all forms of tobacco products, in line with WHO recommendations. Countries underscored the need to enhance monitoring and regulation of smokeless tobacco, areca nut, and novel nicotine products such as electronic cigarettes and nicotine pouches. WHO SEARO reaffirmed its commitment to continue providing technical support, advocacy, and capacity-building assistance to strengthen national implementation of the WHO FCTC and MPOWER measures and support implementation and monitoring of the Framework.

Technical session: Extended and expanded SEAHEARTS milestones 2030: Unhealthy diet

Session	Speakers/Facilitators/ Moderators
Current evidence-based recommendations for population salt/sodium reduction.	Dr Angela de Silva, WHO SEARO
Trans fatty acid regulations: Monitoring, surveillance and implementation- good practices and next steps	Dr Juliawati Untoro, WHO HQ
Progress in Practice: Country Case Highlights	Bangladesh (5 mins, 2 slides each), Thailand, Sri Lanka
Navigating commercial determinants- food industry pushback and solutions (policy compatibility with trade laws, distorted evidence, economic impact)	Dr Kate Robertson, WHO HQ
Group exercise: Identification of priority interventions and/or actions to reach and monitor the SEAHEARTS 2030 on population salt reduction and trans fatty acid elimination and TFA surveillance and enforcement, challenges, opportunities and support needs	
Group exercise continued Discuss and identify next steps to implement the identified priority actions	Country groups Facilitators – Dr Angela de Silva, WHO SEARO Dr Kate Robertson, WHO HQ Dr Juliawati Untoro, WHO HQ

Session objectives:

The sessions aimed to:

- Review progress and challenges in reducing population salt and trans-fat intake in the WHO South-East Asia Region;
- Share country experiences and best practices for policy enforcement and monitoring; and
- Discuss strategies to address commercial determinants of health and industry influence on nutrition policy.

Dr Angela de Silva, Regional Adviser, Nutrition and Health for Development, WHO SEARO presented on the evidence-based recommendations for population salt/sodium reduction, emphasizing that excessive salt consumption remains a key contributor to hypertension and cardiovascular diseases. The presentation highlighted the dual sources of salt intake—discretionary household use and processed or ready-to-eat foods—and the rising contribution of ultra-processed foods to total sodium intake in South-East Asia.

WHO-recommended policy options include reformulation of processed foods, front-of-pack labelling, fiscal policies such as taxation on high-salt and sugar-sweetened foods, and public food procurement standards to promote healthy diets in schools, hospitals, and workplaces. She emphasized aligning salt reduction policies with ongoing salt iodization programmes and the need for a multisectoral approach involving food industry engagement,

strong monitoring systems, and public education campaigns. Challenges identified included limited national data on salt consumption, enforcement capacity gaps, and industry resistance. Nevertheless, several countries have adopted cost-effective, integrated dietary policies, supported by WHO's Global Regulatory Capacity Building Programme and Resolve to Save Lives (RTSL) initiatives.

Dr Juliawati Untoro, Scientist, Department of Nutrition and Food Safety, WHO Headquarters spoke on policy implementation for trans-fat elimination: monitoring and enforcement. Trans fats—commonly found in baked goods, fried snacks, and partially hydrogenated oils—have no known health benefits and substantially increase the risk of heart disease and premature mortality.

She outlined best-practice regulatory measures, including (1) a mandatory national limit of 2 g of iTFA per 100 g of total fat in all foods, and (2) a complete ban on the production or use of partially hydrogenated oils as an ingredient in all foods. Effective monitoring and enforcement require clearly defined authority, adequate human and financial resources, a risk-based inspection system, and laboratory testing of domestic and imported foods. Dr Untoro showcased successful national examples from Saudi Arabia, Lithuania, Singapore, and New York City, illustrating how enforcement mechanisms and industry engagement can achieve high compliance. The presentation also introduced WHO's Validation Programme for Trans-Fat Elimination, recognizing Member States with established legal and monitoring frameworks. Nine countries, including Thailand from the South-East Asia Region, have achieved validation status. Key lessons included the importance of integrating iTFA enforcement within broader food control systems, building laboratory capacity, and providing technical support to small and medium enterprises for product reformulation.

Progress in practice: Country case highlights- The session showcased national experiences from Bangladesh, Sri Lanka, and Thailand in advancing implementation of the SEAHEARTS Milestones 2030 to promote healthier diets through sodium reduction and trans-fat elimination.

Bangladesh reported steady progress in developing sodium benchmarks for packaged foods, supported by WHO and academic partners. A Technical Working Group, comprising government agencies and experts, is drafting the initial sodium benchmarks based on the identification of commonly consumed foods and market survey on sodium content of the packaged foods. The country has also advanced plans for front-of-pack labelling (FoPL), with an inter-ministerial committee and draft design aligned to WHO guidelines. Bangladesh has implemented the Trans-Fatty Acid in Foodstuffs Regulation, 2021, setting a maximum limit of 2% trans-fat in all fats, oil and foodstuffs. Challenges include limited laboratory capacity and inconsistent national food standards, but multisectoral collaboration remains a key enabler.

Sri Lanka highlighted its traffic light front-of-pack labelling system introduced under the Food Act to inform consumers about salt, sugar, and fat content of packaged foods. While enforcement mechanisms exist through food inspectors, compliance remains limited. Identified priorities include refining the FoPL system, setting salt benchmarks for key food categories, and promoting reformulation and public awareness campaigns to enhance consumer understanding.

Thailand presented progress on sodium taxation and public food procurement policies. The proposed sodium tax aims to reduce population salt intake, raise awareness, and encourage food reformulation. Thailand's Guideline on healthy food procurement for public and private sectors—to be launched in 2026—will promote low-sodium, nutritious meals in schools, hospitals, and other institutions. Thailand has also received WHO certificate of Trans Fat Elimination in 2023.

Dr Kate Robertson, Technical Officer (Legal), Public Health Law and Policies, WHO Headquarters, delivered a presentation on Commercial Determinants of Health: Regulation and Response to Industry Pressures. The session highlighted how commercial entities influence public health policy through practices such as lobbying, funding biased research, and exerting political and economic pressure to delay or weaken regulatory action. The importance of legal and regulatory approaches as effective, evidence-based instruments to address health-harming products and practices was highlighted. Such measures must be legitimate, proportionate, and supported by clear regulatory objectives, robust evidence, and transparent governance.

She further discussed strategies to anticipate and counter industry opposition, including early engagement with legal experts, documentation of evidence, and alignment with international guidance. Managing conflicts of interest and ensuring transparency in stakeholder engagement were identified as critical to safeguarding policy integrity.

Drawing on global examples, Dr Robertson presented evidence demonstrating that public health regulations—such as nutrition labelling and marketing restrictions—are cost-effective, promote healthier consumption patterns, and do not adversely impact employment or economic growth. The Chilean experience with front-of-package labelling was cited as a successful model of regulatory effectiveness and economic neutrality.

Country presentations showcased encouraging progress across the WHO South-East Asia Region in implementing national actions for salt reduction and trans-fat elimination, reflecting strong political commitment toward achieving SEAHEARTS 2030 targets. Member States outlined ongoing efforts to develop evidence-based nutrition policies, strengthen legal frameworks, and promote healthier food environments.

Many countries have initiated or updated national salt reduction strategies, incorporating WHO sodium benchmarks, front-of-pack labelling (FoPL), and reformulation of high-salt food products. Several countries, including Bangladesh, are advancing FoPL design and sodium benchmark adaptation through multisectoral technical working groups, while others such as Sri Lanka and Bhutan are promoting healthy canteen guidelines and school-based dietary interventions. Governments are increasingly linking these dietary policies with public food procurement standards to ensure the availability of low-sodium, low-fat foods in schools, hospitals, and workplaces.

On trans-fat elimination, countries reported strengthening legal and regulatory measures in line with WHO's REPLACE framework. Mandatory limits on industrially produced trans-fatty acids (iTFA) have been implemented or are under development, supported by enhanced laboratory capacity and market surveillance. Member States are also exploring consumer education and industry engagement to promote reformulation and compliance.

Thailand presented notable progress in developing a sodium taxation policy, targeting high-sodium packaged foods to reduce consumption and incentivize reformulation. The initiative, led by the Ministry of Finance and the National Sodium Reduction Committee, demonstrates innovative use of fiscal policy to create an enabling environment for healthy diets. Thailand has also advanced public food procurement guidelines for healthy meal standards across schools, hospitals, and government institutions, and has fully enforced its national ban on partially hydrogenated oils since 2018, earning WHO's Certificate of Trans-Fat Elimination.

Across the Region, countries identified common challenges such as limited monitoring capacity, resource constraints, and resistance from industry stakeholders. They emphasized the need for technical support to enhance laboratory testing, enforcement, and cross-sector coordination. Overall, the presentations emphasized the Region's growing momentum toward

creating healthier food systems through integrated, multisectoral approaches—anchored in strong regulatory frameworks, policy innovation, and public awareness.

Discussions on unhealthy diets emphasized the critical need to reduce population-level intake of sodium and eliminate industrially produced trans fatty acids. Member States committed to scaling up policy implementation and enforcement through evidence-based legislative and regulatory measures, including food reformulation, procurement standards, and healthy cafeteria policies. The importance of lab capacity strengthening, stakeholder engagement, and behavior change communication for promoting healthy dietary practices was also highlighted. WHO assured continued support through high-level advocacy, technical guidance, and capacity-building to address commercial determinants of unhealthy diets and to ensure monitoring and enforcement of sodium and trans-fat policies.

Technical session: Comprehensive cancer prevention and management

Session	Speakers/Facilitators/Moderators
Status of implementation of the WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management.	Dr Bishnu Rath Giri, WHO SEARO
Leveraging the special initiatives on cancer control to accelerate implementation of actions:	
Cervical cancer elimination initiative	Dr Prebo Barango, WHO HQ *(virtual)* Smiljka de Lussigny, UNITAID (*virtual)*
Global breast cancer initiative	Dr Mary Nyangasi, WHO HQ *(virtual)*
Global initiative for childhood cancer	Dr Roberta Ortiz, WHO HQ *(virtual)* Ms. Y Ravindran/Dr Catherine Lam, St Jude Children's Research Hospital *(virtual)*
imPACT reviews and Rays of hope initiative	Ms. Kimberley Frolov-Roessler/ Mr. Karagu Maina *(virtual)* International Atomic Energy Agency
South-East Asia Cancer Grid	Dr Pramesh CS, Tata Memorial Hospital *(virtual)*
Group presentations National priority actions and experience from country	

Session objectives:

- Updated status of implementation of Regional cancer strategy
- National priority actions to accelerate the implementation of cancer strategy and programs leveraging on the global initiatives on cancer identified.

The third day of the Regional Workshop focused on advancing comprehensive cancer control in the WHO South-East Asia Region. The objective of the session was to review the progress made by Member States in implementing national cancer control programmes, align ongoing initiatives with the WHO regional strategy, and identify opportunities for collaboration and capacity-building across countries. The session brought together global partners and regional experts to discuss policy, implementation challenges, and innovative approaches to improving access to prevention, early detection, diagnosis, timely treatment, and palliative care for priority cancers.

Dr Bishnu Rath Giri, Technical Officer for Cancer Control at WHO SEARO, presented an overview of the cancer situation and progress in implementing the WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management (2024–2030). He noted that the Region continues to face a growing cancer burden, with an estimated 1.9 million new cases and 1.3 million deaths in 2022 and cancer cases and deaths projected to nearly double by 2050 if current trends were to continue. He highlighted that the mortality-to-incidence ratio of 65% is almost twice that of high-income countries (36%), reflecting significant gaps in early detection, diagnosis and quality assured treatment services. He also provided an overview on the Regional Strategy, approved at the 77th Regional Committee meeting, proposes seven strategic priorities and a monitoring framework comprising 19 indicators to be reported biennially. Dr Giri emphasized that while all countries have national programs for cervical cancer screening and several for breast and oral cancer screening, coverage remains low in most. He also shared that while tertiary care centre for cancer is said to be generally available in most countries, diagnostic and treatment services including surgery and chemotherapy are generally available in lesser number of countries with radiation therapy being generally available only in six countries. He noted that five countries have operational population-based cancer registry with varying coverage, with the registry of one country covering the entire population. He noted, however, that despite these challenges, promising developments were reported, including Bhutan's comprehensive cancer registry, Sri Lanka's integration of imPACT review recommendations into its national strategy, and Thailand's expansion of self-sampling methods for cervical screening.

A joint presentation by Dr Prebo Barango from WHO and Ms Smiljka Lussigny from UNITAID provided an overview of the WHO Global Strategy for Cervical Cancer Elimination. The speakers highlighted that elimination is achievable within the century if countries accelerate progress towards the 90–70–90 targets—90% of girls fully vaccinated with HPV vaccine by age 15, 70% of women are screened with a high-performance test by 35 years of age and again by 45 years, and 90% of women identified with cervical disease receive treatment by 2030. The speakers shared the fact that globally at least one dose of HPV vaccine has been received by 31% girls, and inequities remain pronounced. The single-dose HPV vaccination schedule, endorsed by WHO, offers a cost-effective and scalable approach to improving coverage, particularly in low- and middle-income countries. The importance of transitioning from opportunistic to population-based screening and promoting HPV DNA testing and self-sampling as key tools for scale-up was stressed upon. Dr Prebo highlighted that innovative low-cost treatment technologies such as portable thermal ablation and LEEP devices offer effective solutions for increasing access to care in underserved settings and collaboration between countries can enhance capacity to expand services for treatment. Ms Lussigny shared examples of partners' support to countries in HPV vaccination and cervical cancer screening, also emphasizing the role partners have played in reducing the cost of critical supplies such as HPV DNA tests. The speakers highlighted WHO and partners' readiness to support Member States in developing policies, training health workforces, and fostering collaboration to share lessons and accelerate impact. It was emphasized that partners are ready for supporting the priority interventions led by the country.

Dr Mary Nyangasi, Technical Lead for the WHO Global Breast Cancer Initiative (GBCI), presented on the Global Breast Cancer Initiative. She emphasized that breast cancer remains the most common cancer among women in 85% of countries and the leading cause of female cancer deaths worldwide. The GBCI aims to achieve a 2.5% annual reduction in global mortality by 2040 through three pillars: early diagnosis, timely diagnostic evaluation, and comprehensive treatment. Dr Nyangasi highlighted that success in high-income countries has been driven by strong health systems and patient navigation models that ensure timely and coordinated care. She also highlighted the importance of population-based cancer registries for tracking outcomes and guiding policy decisions.

The session on childhood cancer, presented by Dr Roberta Ortiz from WHO Headquarters, Dr Catherine Lam and Ms Yadurshini Raveendran from St. Jude Children's Research Hospital, highlighted the WHO Global Initiative for Childhood Cancer (GICC) and the CureAll Framework for implementation. The speakers emphasized that childhood cancer remains a leading cause of death among children aged 0–19 years, with nearly 90% of cases occurring in low- and middle-income countries, where survival rates often remain below 30%,

in contrast to over 80% in high income countries. Launched in 2018, the GICC aims to achieve at least 60% survival by 2030 and reduce suffering by integrating childhood cancer care within universal health coverage (UHC) and strengthening national systems for diagnosis, treatment, and monitoring. The speakers presented the CureAll Framework, that guides countries through four pillars—investing in centres of excellence, UHC inclusion, promoting access to essential medicines, and robust monitoring systems—supported by enablers viz. advocacy, leveraged financing, and linked polices and governance.

One of the key achievement under this initiative is the Global Platform for Access to Childhood Cancer Medicines (GPACCM), which ensures the sustainable supply of quality-assured cancer medicines to LMICs. Ms Yadurshini shared that since its launch, the platform has delivered over 2.4 million doses of medicine to six countries, benefiting more than 5,000 children across 30 hospitals, with expansion planned for additional countries. She also shared that the initiative has already driven tangible policy impact: Nepal, the participant country in first cohort from the region, introduced free treatment for children with cancer up to age 14 leveraging on the support from GPACCM. The speakers in the session emphasized how CureAll is catalysing national action, building regional cooperation, and ensuring that childhood cancer care becomes an integral part of health system strengthening and UHC across the South-East Asia Region.

The International Atomic Energy Agency (IAEA) presented the Rays of Hope initiative and the imPACT Review process. Ms. Kimberley Frolov-Roessler described how the Rays of Hope initiative, launched in 2022, addresses inequities in access to radiation medicine by supporting countries to establish and strengthen diagnostic imaging, radiotherapy, and nuclear medicine services. Dr Alfred Karagu presented on how the imPACT Review mechanism provides countries with a comprehensive assessment of their cancer control capacity and identifies priority interventions for investment. He also shared that since 2005, 122 imPACT Reviews have been conducted in 100 Member States, serving as the foundation for the development or revision of National Cancer Control Plans and the mobilization of resources for implementation.

Dr C. S. Pramesh presented the South-East Asia Cancer Grid (SEACanGrid), a regional collaboration led by WHO SEARO and Tata Memorial Hospital, Mumbai, aimed at reducing disparities in cancer care through shared learning, research, and capacity building. Drawing inspiration from India's National Cancer Grid and South-East Asia Regional Childhood Cancer Network (SEAR-CCN), SEACanGrid connects leading cancer institutions across the Region to promote uniform standards of care, quality assurance, and context-specific research. Key activities include capacity building, virtual tumor boards, online and blended learning programmes, and quality improvement initiatives. He mentioned that by fostering digital collaboration and harmonized practices, SEACanGrid seeks to strengthen cancer systems and advance equitable, high-quality cancer care across South-East Asia.

The group presentation session served as a platform for participating Member States to share country experiences, identify priority gaps, and outline strategic actions for advancing cancer control over the next five years. Countries presented their priority gaps and corresponding actions identified for addressing them following structured group discussions that focused on assessing the current status of cancer prevention and control, identifying

system gaps, and ranking priorities for action. Each country team highlighted major challenges and solutions aligned with the strategic priority of the WHO South-East Asia Regional Strategy for Comprehensive Cancer Prevention and Management.

Common challenges faced by many countries included limited coverage of population-based screening for cervical, breast, and oral cancers; inadequate early detection and referral mechanisms; shortages of trained oncology and pathology personnel; fragmented data systems; and constrained access to radiotherapy and chemotherapy services, particularly in peripheral areas. Countries also emphasized the need to strengthen cancer registries, as well as the integration of cancer care within broader noncommunicable disease (NCD) and primary health care frameworks.

Cancer situation assessment was identified as priority action by Bangladesh team. Drafting of national cancer plan was a priority for Timor-Leste. Cancer registry strengthening was prioritized by Bangladesh, Bhutan, Thailand and Timor-Leste. Increasing the coverage and quality assurance in cervical screening was priority identified by Bhutan, Maldives, Nepal, and Thailand. Sri Lanka had expanding oral cancer screening as one of its priority. Strengthening pathology diagnostics for cancer was a priority identified by Sri Lanka and Timor-Leste. India team included expansion of Day-care chemotherapy centres at district level among its priority actions. Several countries identified workforce capacity building and partnerships for diagnostics and tele-oncology as critical next steps.

Country teams also presented examples of promising practices and innovations. India and Thailand shared experiences on scaling up district-level chemotherapy services and digital health innovations for screening and follow-up. Thailand shared the experience of expanding self-sampling for HPV DNA testing. Bhutan and Sri Lanka highlighted the integration of cancer control activities into universal health coverage (UHC) and the use of imPACT reviews to guide national planning. Nepal described policy reforms that expanded free cancer care for children and efforts to strengthen cancer registries, while Bangladesh presented multisectoral collaborations for community-based awareness and screening.

The discussions highlighted shared regional priorities — enhancing screening coverage, ensuring equitable access to diagnosis and treatment, investing in workforce development, and improving data-driven governance. Despite varying resource contexts, all countries reaffirmed their commitment to align national strategies with WHO's Regional Strategy for comprehensive cancer prevention and management, regional initiatives such as SEACanGrid and the Global Initiatives for Cervical, Breast, and Childhood Cancers.

For cancer control, participants reflected on the technical sessions and group discussions, reaffirming the regional priorities of scaling up population-based screening for cervical, breast, and oral cancers, improving access to early diagnosis and treatment, expanding cancer registries, and ensuring the availability of essential medicines and radiotherapy services. The role of regional collaborations such as SEACanGrid, and global partnerships including the Cervical Cancer Elimination Initiative, the Global Breast Cancer Initiative, the Global Initiative for Childhood Cancer, and the IAEA's Rays of Hope and imPACT Reviews, were recognized as instrumental in advancing equitable access and building technical capacity across Member States.

Technical session: Framework for monitoring SEAHEARTS 2030 milestones

Session	Speakers/Facilitators/Moderators
Framework for monitoring SEAHEARTS 2030 milestones	Dr Nalika Gunawardena, WHO SEARO
Best practice dashboards in health facilities: facilitating tracking the progress to reach national targets for SEAHEARTS	Dr Pradeep Joshi, WHO SEARO A nominated official, Bangladesh
Using routine health information systems to monitor SEAHEARTS 2030 milestones	Dr Amani Siyam, WHO SEARO

Session objectives:

The session aimed to

- Present and discuss the draft SEAHEARTS 2030 Monitoring Framework, including operational definitions, metadata, and data requirements for each milestone;
- Review mechanisms for tracking national progress on hypertension and diabetes treatment coverage and control, tobacco control, salt reduction, and trans-fat elimination;
- Explore the role of digital innovations and routine health information systems (RHIS) in data collection, analysis, and reporting; and
- Gather feedback from Member States to refine the monitoring framework and identify next steps for country implementation.

Dr Nalika Gunawardena presented the framework for monitoring SEAHEARTS 2030 milestones, which defines operational indicators for each of the five milestones: hypertension management and control, diabetes management and control, tobacco control (MPOWER implementation), salt/sodium reduction, and trans-fat elimination. She explained that these indicators align with the WHO Noncommunicable disease facility-based monitoring guidance and global benchmarks, including the Global Tobacco Epidemic Report, the WHO Sodium Scorecard, and the Trans-Fatty Acid Scorecard.

The process indicators to monitor progress in establishing national targets and roadmaps for hypertension and diabetes coverage and control were also presented. The frequency of reporting of the data was proposed as annually by national NCD focal points with data presented through the SEAHEARTS Monitoring Platform and digital dashboard, which will function as a regional hub for tracking milestones, sharing case studies, and disseminating best practices.

Presenting the best practice dashboards in health facilities to visualize and track core hypertension and diabetes indicators of SEAHEARTS 2030 at national and facility levels Dr Pradeep Joshi discussed salient features of digital dashboards. Dashboards were described as tools to transform data into actionable insights through simple visualizations, such as trend lines, color-coded alerts, and summary indicators. These tools help program managers and policymakers identify bottlenecks, track patient outcomes, and take evidence-based corrective action.

Following country examples of effective use of digital innovations were also demonstrated:

- India's NCD Portal and Dashboards which allow real-time tracking of hypertension and diabetes indicators from community to national levels, with integrated modules to visualize screening, referral, treatment, control and follow-up implemented in more than 120,000 health facilities. Based on the analysis of indicators, the portal generates workplan for providers to assist them in taking timely action for improving the quality of care.
- Bangladesh's Simple App, implemented across 495 Upazila Health Complexes, supports continuity of care for over half a million hypertension patients and integrates with DHIS2 for national reporting. The dashboard provides high level, simplified summary to health care workers to track the enrolment, treatment coverage, control, drug stock outs along with comparison of other health complexes to guide programme managers to define actions for improvement.

- Timor-Leste's NCD e-Tracker, embedded within the DHIS2 platform, enables patient-level data capture and disaggregation down to village level, improving program accountability and monitoring. The e-tracker is integrated in the existing platform of Integrated case-based surveillance (TB, HIV and Malaria) to optimally utilize the existing resources and capture key information from NCD register to e- tracker. The dashboard visualization front end is under development but will incorporate the features of best practice dashboard.

These examples illustrated how user-friendly, interoperable digital systems can enhance data-driven decision-making and accelerate achievement of SEAHEARTS targets.

Dr Amani Siyam, Regional Adviser (Health Information Systems), WHO SEARO emphasized the need to embed SEAHEARTS indicators within the Health Information System (HIS)/ Routine Health Information System (RHIS) to promote sustainability and regular data use for decision-making. She outlined the role of RHIS as a core component of national health information systems, enabling continuous monitoring of service delivery, resource use, and health outcomes. She presented the SCORE for Health Framework, a WHO tool to assess data system maturity and identify investment priorities for digital health transformation. Emerging opportunities under the Global Digital Health Certification Network (GDHCN) and Verifiable Digital Health Wallets were introduced as part of WHO's global digital ecosystem for secure, interoperable, and verifiable data exchange to strengthen NCD monitoring and evaluation capacities.

Participants reviewed the draft SEAHEARTS 2030 Monitoring Framework, providing inputs on the indicator definitions, data requirements, and reporting feasibility. Countries discussed the practicality of annual data collection, identified areas where reporting may not be feasible, and recommended improvements to process indicators for hypertension, diabetes, and dietary risk monitoring. The feedback collected will inform the finalization of the framework and guide WHO technical assistance for country adaptation. The revised SEAHEARTS monitoring framework can be accessed at https://iris.who.int/handle/10665/384720

Closing session

The concluding session of the Regional Workshop to advance NCD prevention and control in the WHO South-East Asia Region focused on consolidating the outcomes of the three-day deliberations and reaffirming the regional commitment to achieving the SEAHEARTS Milestones 2030. WHO technical staff presented an overview of the National Roadmaps and priority actions submitted by Member States to reach the SEAHEARTS 2030 milestones and priority cancer actions.

WHO reaffirmed its continued technical support to Member States to fulfill the following requests:

- Provide guidance on clinical audit design and programme review of hypertension and diabetes treatment coverage and control quality improvement.
- Technical guidance on updating national hypertension/ diabetes protocols.
- Technical assistance to improve accessibility of insulin in primary health care.
- Strengthen support on procurement of validated BP devices and diagnostics, and regular updates on WHO-prequalified health technologies for diagnosis of diabetes and hypertension.
- Provide guidance on linking laboratories at higher levels with PHC to improve accessibility of testing.
- Support in using the morbidity-based forecasting tools for effective quantification of medicines, diagnostics, and reagents.
- Provide PEN/HEARTS-based training modules, Training of Trainers packages, supportive supervision tools, and clinical mentorship resources.
- Develop training modules for monitoring key NCD indicators; support standardization of data collection and reporting; and provide review formats, feedback templates, and technical summaries to periodically review the progress on national commitments.
- Provide strategic leadership, advocacy, and facilitation of the Regional Framework on smokeless tobacco, areca nut, and novel nicotine products.
- Offer evidence-informed tools and best practices; support training and capacity building; facilitate youth tobacco surveillance; and promote multi-country research on smokeless tobacco, areca nut, and MPOWER implementation.
- Support high-level policy advocacy and provide technical guidance on addressing commercial determinants and conflicts of interest.
- Assist with evidence generation, regulatory development, policy implementation, and strengthening laboratory capacity for food monitoring.
- For cancer control- Provide quality assurance frameworks, guidelines, case studies, capacity-building support, coordination of imPACT review missions, support for registry strengthening, and technological assistance based on country-specific requests.

The workshop concluded with Member States reaffirming their collective determination to translate strategic commitments into tangible outcomes, accelerate progress across the SEAHEARTS Milestones, and advance equity in NCD prevention and control. Countries highlighted the importance of sustained WHO support across technical, regulatory, programmatic, and capacity-building domains to help strengthen health systems and improve population health across the WHO South-East Asia Region.

Annex 1

Programme

Day 1 – 28 October 2025		
Time (hrs.) IST	**Inaugural session**	
0830–0900	Registration	
0900–0910	Opening remarks	WHO SEARO
0910–0930	Opening session	
0930–0950	SEAHEARTS 2025: achievements, and lessons learnt	Dr Nalika Gunawardena, WHO SEARO
0950–1000	Global Commitments on NCDs: Turning the Tide Together	Dr Alarcos Cieza, WHO HQ
1000–1020	Introduction of the participants	
1020–1040	*Group photo and Healthy break*	
	Session	**Speakers /Facilitators/ Moderators**
	Extended and expanded SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control	
1040–1115	Regional SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control	Dr Pradeep Joshi, WHO SEARO
1115–1230	A panel discussion on overcoming common challenges to reach the SEAHEARTS 2030 milestones	
	Avoiding stock-outs of medicines and technologies through realistic forecasting	Mr Bashier Enoos, WHO HQ *(virtual)* A nominated official, India
	Approaches to promote use of statins in primary health care level	Dr Jeyaraj Durai Pandian, WHO CC for Stroke Care, Christian Medical College, Ludhiana, India A nominated official, Sri Lanka
	Improving treatment adherence for hypertension through single-pill combinations	Dr Kouamivi Agboyibor, WHO HQ *(virtual)* A nominated official, Bhutan
	Enhancing access to HbA1c to monitor glycaemic control at primary health care level	Dr Aqsha Azhary Nur, WHO HQ *(virtual)* A nominated official, Timor-Leste

	Ensuring availability of validated BP measuring devices in primary care level	Dr Swagata Kumar Sahoo, Resolve to Save Lives A nominated official, Nepal
	Question and answers	
1230–1330	*Food for thought – lunch break* A special session- Presentation of the protocol of a randomised control trial on a diet related intervention for better glycaemic control among diabetics in Bhutan	
1330–1500	**Group work** Framing country targets and designing national roadmap to reach SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control	Country groups
	Healthy break	
1500-1700	**Country presentations** and discussions	Nominated officials
Day 2 – 29 October 2025		
Time (hrs.) IST	**Session**	**Speakers/Facilitators/ Moderators**
0900–0905	Recap-day 1	
	Extended and expanded SEAHEARTS milestones 2030: tobacco control	
0905–0920	Regional strategic framework for combating smokeless tobacco, novel nicotine products and areca nut in the WHO South-East Asia Region (2025-2030)	Dr Jagdish Kaur, WHO SEARO
0920–0940	Open discussion	
0940–1000	*Healthy break*	
1000–1100	**Country presentations-** identification of priority interventions to reach and monitor the SEAHEARTS 2030 on tobacco control	Nominated officials of all countries
1100–1115	Updated status of WHO MPOWER implementation – gaps and challenges	Dr Hebe Gouda, WHO HQ *(virtual)*
1115–1130	Questions and answers	
1145–1200	Tobacco cessation programme in Rajasthan	WCO India
1200–1215	Tobacco products testing – updated evidence	NIMHANS Bangalore
1215–-1230	Policies for prevention and control of smokeless tobacco	WHO FCTC Knowledge Hub on Smokeless Tobacco
1230–1315	*Food for thought – lunch break*	
	Extended and expanded SEAHEARTS milestones 2030: unhealthy diet	

1315–1325	Current evidence-based recommendations for population salt/sodium reduction.	Dr Angela de Silva, WHO SEARO
1325–1345	Trans fatty acid regulations: Monitoring, surveillance and implementation- good practices and next steps Question and answers	Dr Juliawati Untoro, WHO HQ
1345–1405	Progress in Practice: Country Case Highlights	Bangladesh (5 mins, 2 slides each), Thailand, Sri Lanka
1405–1425	Navigating commercial determinants- food industry pushback and solutions (policy compatibility with trade laws, distorted evidence, economic impact) Question and answers- 10 mins	Dr Kate Robertson, WHO HQ
1425–1500	**Group exercise:** Identification of priority interventions and/or actions to reach and monitor the SEAHEARTS 2030 on population salt reduction and trans fatty acid elimination and TFA surveillance and enforcement, challenges, opportunities and support needs	
1500–1515	*Healthy break*	
1515–1615	**Group exercise** continued Discuss and identify next steps to implement the identified priority actions	Country groups Facilitators- Dr Angela de Silva, WHO SEARO Dr Kate Robertson, WHO HQ DR Juliawati Untoro, WHO HQ
1615–1700	**Group presentations**	
Day 3 – 30 October 2025		
Time (hrs.) IST	**Session**	**Speakers /Facilitators / Moderators**
0900–0905	Recap-day 2	
	Comprehensive cancer prevention and management	
0905–0920	Status of implementation of the WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management.	Dr Bishnu Rath Giri, WHO SEARO
0920–1020	Leveraging the special initiatives on cancer control to accelerate implementation of actions:	
	Cervical cancer elimination initiative	Dr Prebo Barango, WHO HQ *(virtual)* Smiljka de Lussigny, UNITAID (*virtual)*
	Global breast cancer initiative	Dr Mary Nyangasi, WHO HQ *(virtual)*

	Global initiative for childhood cancer	Dr Roberta Ortiz, WHO HQ *(virtual)* Ms. Y Ravindran/Dr Catherine Lam, St Jude Research Hospital *(virtual)*
	imPACT reviews and Rays of hope initiative	Ms. Kimberley Frolov-Roessler / Mr. Karagu Maina *(virtual)* International Atomic Energy Agency
	South-East Asia Cancer Grid	Dr Pramesh CS, Tata Memorial Hospital *(virtual)*
1020–1040	*Healthy break*	
1040–1130	**Group exercise** National priority actions to accelerate the implementation of the WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management (2024-2030)	Country groups
1130–1230	**Group presentations** National priority actions and experience from country	
1230–1330	*Food for thought – lunch break*	
	Framework for monitoring SEAHEARTS 2030 milestones	
1330–1345	Framework for monitoring SEAHEARTS 2030 milestones	Dr Nalika Gunawardena, WHO SEARO
1345–1410	Best practice dashboards in health facilities: facilitating tracking the progress to reach national targets for SEAHEARTS	Dr Pradeep Joshi, WHO SEARO A nominated official, Bangladesh
1410–1425	Using routine health information systems to monitor SEAHEARTS 2030 milestones	Dr Amani Siyam, WHO SEARO
1425–1445	**Group work**: feedback on the framework for monitoring SEAHEARTS 2030 milestones	
	Concluding session	
1445–1515	Next steps and participant reflections	
	Healthy break and conclusion	

Annex 2

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Annex 3

Opening remarks – Officer in Charge, WHO South-East Asia Region

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Officials and dignitaries from our Member States

Partners, colleagues and friends

Good morning to you all.

On behalf of WHO, I welcome all distinguished delegates and participants to this important regional workshop titled From *Strategy to Actions: Prioritizing NCD Prevention & control in the WHO South-East Asia Region*.

My thanks to the Government of India and the Government of Rajasthan for hosting us here in historic Jaipur.

This workshop comes at an important moment.

Two weeks ago, the 78th Session of the Regional Committee deliberated on two major agenda items:

- The Regional Strategic Framework for combating smokeless tobacco, novel nicotine products, and areca nut; and
- SEAHEARTS – our landmark initiative for cardiovascular disease, combining risk factor reduction with strengthening of hypertension and diabetes management.

NCD prevention and control is also a global priority- last month, the UN General Assembly held its Fourth High-Level Meeting on NCDs and Mental Health.

NCDs remain one of the greatest public health and development challenges of our time.

In our region alone, they were the cause of over 9 million deaths in 2021, half of them premature. Cardiovascular diseases and cancer account for the highest proportion of these premature deaths—49% and 66% respectively.

Ladies and gentlemen,

Target 3.4 of the Sustainable Development Goals commits the world to reduce premature mortality from NCDs by one-third by 2030.

Unfortunately, according to the Global Health Estimates 2021, we are not on track.

In our region, premature NCD mortality has remained stubbornly high since 2000, putting us at serious risk of missing the target.

It is clear that we urgently need to move from strategy to action.

We must accelerate actions on two important fronts:

- First, by implementing cost-effective policy, fiscal and regulatory measures to reduce key risk factors for NCDs
- Second, by strengthening primary health care to deliver NCD services, with a focus on good patient outcomes.

The extended and expanded *SEAHEARTS Milestones 2030*, endorsed by the Regional Committee earlier this month, is a roadmap for our region for this acceleration. I congratulate our Member States for this ambitious, yet achievable, commitment.

This workshop is an important step in fulfilling our mandate to technically support our countries —by discussing challenges and formulating national roadmaps to translate WHO technical packages into actionable steps to achieve these 2030 milestones.

You will also review the implementation progress of the *Regional Strategy for Comprehensive Cancer Prevention and Management (2024–2030)* and will support Member States in designing national priority actions.

Ladies and gentlemen,

This is our moment to learn from one another, to inspire each other, and to turn strategy into action.

WHO stands firmly with you, and I wish you a productive and successful workshop.

Thank you.

Annex 4

Seventy-eighth Regional Committee Decision on Extending and expanding the Regional Milestones of SEAHEARTS [SEA/RC78(7)]

World Health Organization
South-East Asia Region

DECISION

OF THE

WHO REGIONAL COMMITTEE FOR SOUTH-EAST ASIA

SEA/RC78(7)

Extending and expanding the Regional Milestones of SEAHEARTS: an initiative to reduce CVD burden in the WHO South-East Asia Region

The Regional Committee,

HAVING CONSIDERED the updates on specific Regional Committee resolutions and documents presented to its Seventy-eighth Session, including the Progress Report on its resolution SEA/RC76/R5 titled '*SEAHEARTS: Accelerating prevention and control of cardiovascular diseases in the WHO SE Asia Region*'.

NOTING the progress made over the implementation of SEAHEARTS interventions including on the four Regional Milestones1 for 2025, and the suggested way forward therein,

RECOGNIZING that the burden of cardiovascular diseases in the South-East Asia Region is expected to continue to increase due to population growth, ageing and high prevalence of risk factors,

RECOGNIZING FURTHER that the Region still faces considerable challenges in addressing the major CVD risk factors, particularly uncontrolled hypertension, along with diabetes, tobacco use, unhealthy diet (including high salt intake and consumption of trans-fatty acids), and physical inactivity; that the interventions aimed at achieving the expanded Milestones require comprehensive and integrated approaches; and that they have strong correlations and complementarities with the interventions under the Global NCD Targets as well as the SDG Targets set for 2030,

1 SEAHEARTS: an initiative to reduce CVD burden in the WHO South-East Asia Region. New Delhi: WHO Regional Office for South-East Asia. https://cdn.who.int/media/docs/default-source/searo/ncd/seahearts-reduce-cardiovasculardisease-burden-in-the-sear.pdf - accessed 2 September 2025.

DECIDES:

1. **TO EXTEND** the timeline of the four Regional Milestones of the SEAHEARTS Initiative for another 5 years, till 2030; and

2. **TO REQUEST** the Regional Director to:

   (a) convene technical consultation(s) with Member States and key stakeholders to develop the proposed extended and expanded regional SEAHEARTS Milestones for 2030, encompassing tobacco control; salt reduction; elimination of trans-fatty acids; and most urgently, improved detection, treatment and control of hypertension and diabetes as primary risk factors for cardiovascular diseases;

   (b) support Member States in developing national and subnational roadmaps with targets for the implementation of WHO technical packages, HEARTS[2] (technical package for CVD management in primary health care), MPOWER[3] (measures to reduce tobacco demand under the WHO Framework Convention on Tobacco Control (FCTC), SHAKE[4] (technical package for salt reduction) and REPLACE[5] (technical package for eliminating industrially produced trans-fatty acids), in order to achieve the SEAHEARTS 2030 milestones with particular emphasis on reducing uncontrolled hypertension through population-level and clinical interventions; and

   (c) report the progress, achievements and challenges on the implementation of this decision to the Regional Committee sessions in 2026, 2028 and 2030.

2 HEARTS: a technical package for cardiovascular disease management in primary health care. Geneva: World Health Organization, 2020. https://www.who.int/publications/i/item/9789240001367

3 MOPWER: a technical package for tobacco control. Geneva: World Health Organization, 2023. https://www.who.int/initiatives/mpower

4 SHAKE: a technical package for salt reduction. Geneva: World Health Organization, 2016. https://apps.who.int/iris/bitstream/handle/10665/250135/9789241511346-eng.pdf;sequence=1

5 REPLACE: an action package to eliminate industrially produced trans-fatty acids. Geneva: World Health Organization, 2021. https://www.who.int/publications/i/item/9789240021105

SEA HEARTS

World Health Organization
South-East Asia Region

https://iris.who.int/server/api/core/bitstreams/a87a3112-838b-4800-8fb2-6cd767fbeb5c/content

Annex 5

Summary of country presentations on national roadmaps to achieve the SEAHEARTS 2030 milestones and priority actions for cancer control

National roadmap to reach SEAHEARTS milestones 2030: Hypertension and diabetes treatment coverage and control

National commitment

Country	Hypertension		Diabetes	
	Total number of people to be placed on protocol-based management by 2030	Number of people having blood pressure under control by 2030	Total number of people to be placed on protocol-based management by 2030	Number of people having good control of glycaemia by 2030
Bangladesh	4,341,000	2,604,000	5,380,000	2,690,000
Bhutan	72,000	43,000	22,000	17,000
DPR Korea	2,012,524	1,207,514	1,414,934	707,467
India	80,000,000	48,000,000	50,000,000	20,000,000
Maldives	25,000	12,500	15,000	7,500
Nepal	700,000	400,000	400,000	160,000
Sri Lanka	1,000,000	547,800	1,000,000	573,300
Thailand	7,500,000	5,700,000	3,700,000	2,800,000
Timor-Leste	87,000	52,000	28,000	14,000

Country	Themes		
	National commitment towards SEAHEARTS 2030 and announcement	Measures to improve early detection of HTN and DM and register them for care	Standard treatment protocols for HTN and DM
Bangladesh	Increase HTN and DM screening and management to reach the SEAHEARTS target; NCD service delivery at PHCs include in the revenue budget	Screening of HTN & DM at PHCs under local government division, DGFP; mandatory screening for all government employees under the healthy workplace initiative	Integrated using of standard treatment protocols for HTN and DM at PHCs under LGD
Bhutan	Sensitize policy makers and sub-national authorities on SEAHEARTS 2030 targets and national commitments	Targeted screening for district with less than 75% coverage; All health facilities to conduct opportunistic screening	Include insulin in the national diabetes treatment protocol, insulin supply chain strengthening, and training resources for PHC staff.
India	Finalize India's SEAHEARTS 2030 hypertension and diabetes targets; announce nationally;	Further strengthening of screening for 30+ population through Ayushman Arogya Mandirs (AAMs) under NP-	Update and disseminate national standard treatment protocols (STPs) for hypertension and diabetes

Country	Themes		
	National commitment towards SEAHEARTS 2030 and announcement	**Measures to improve early detection of HTN and DM and register them for care**	**Standard treatment protocols for HTN and DM**
	disseminate to States/ UTs with goal setting workshops; Dissemination of targets to States/ UTs with State-specific goal-setting workshops	NCD program for early detection (rural to urban), ensuring availability of validated BP apparatus and glucometer strips	aligned with WHO HEARTS technical package. States/UTs to implement; capacity-building for MOs, CHOs and nurses on STPs. (National Health Mission identifies training institute for Pool of master trainers)
Maldives	Proposed country commitment for SEAHEARTS 2030 is accepted	Models of screening at workplaces is in operation; contextualised models of screening in urban settings are in operation; contextualised models of screening of other vulnerable population groups are in operation	Protocol for HTN include single combination pills
Nepal	SEAHEARTS 2030 commitment is accepted; announce the target; advocate for required budget and program support underway; sensitize subnational levels to set their targets.	Screening programs, engagement of FCHVs, strengthen DIHS, resource allocation	Nationally protocols for HTN and DM at PHC available-update the protocol; capacity building to reduce treatment inertia
Sri Lanka	75% of hypertensives diagnosed: opportunistic and community-based screening; public awareness campaign & empowerment of community; HLC screening. 90% treated: strengthen primary care; streamline procurement; patient support & reminders; task shifting; adequate equipment and lab facilities; lifestyle modification. 75% controlled: implement protocol-based management in all PMCIs, maintain optimum titration%, strengthen follow-up. 75% of DM patients diagnosed, 90% diagnosed treated, 50% diabetic patients are under control. Primary care facilities in all districts use protocol-based management; Provinces/ districts/Municipalities/ Townships are sensitized to set-up subnational level targets.	Introduce opportunistic OPD screening and workplace screening models; continue urban areas and community-based screening and outreach clinics; obtain data from private sector; continued screening for high-risk/ vulnerable populations	Update protocol for hypertension and diabetes including statins to treatment protocols, introduce protocol-based management to patients already on treatment; dose intensification/titration for PHC staff, insulin included and used at PHCs; protocols implemented in secondary/tertiary care and private sector.

Country	Themes		
	National commitment towards SEAHEARTS 2030 and announcement	**Measures to improve early detection of HTN and DM and register them for care**	**Standard treatment protocols for HTN and DM**
Thailand		Nationwide hospital-based screening– to reduce under diagnosis; hypertension control change package to improve HTN management and control; improving guideline of New Thai CVD Score (CVD risk score for early detection)	DM Remission (intensive lifestyle modification) and treatment according to Thai CPG.
Timor-Leste	SEAHEARTS 2030 commitment accepted; national and subnational targets will be announced and disseminated to municipal and sub-municipal program managers. Advocate for programmatic and budgetary support by meeting with heads of INFP, DNEM, and DNFM.	Expand triage areas for opportunistic screening of diabetes and hypertension among adults to additional municipalities with established referral pathways; develop operational guidelines for bidirectional screening of DM and HIV/AIDS and TB programs; develop operational guidelines for GDM.	To scale up the newly developed protocol for DM and HTN in all the municipalities which also has statins and aspirin use according to history of CVD; to develop SOP for the private sector to implement the National DM and HTN guidelines

Country	Themes			
	Access to essential medicines and devices	**Team-based care and task sharing**	**Promote continuity of services through patient centred services**	**Information systems to monitor patient and programmatic outcomes**
Bangladesh	All Essential NCD medicine included in the NEML including statins, Insulin & oral morphine; forecasting model based on mixed based system/ morbidity system piloted & integrated into PHC setting.			Ensure patient registered and monitoring through Simple App in all 495 PHCs; introduce open MRS (medical record system)
Bhutan	Ensure all essential NCD medicines are available in blister packs at all health facilities; equip all PHCs with automated, professional-grade, and clinically validated BP measurement devices.			Include diabetes tracking system in the NCD digital dashboard
India	Ensure availability of essential NCD medicines as per treatment protocols at all Ayushman Arogya Mandirs. The forecasting will be based on morbidity using ready reckoner tools; expand access to	Conduct training on team-based NCD management and task-sharing for CHOs and ANMs; defining defined roles and		Strengthen NP-NCD portal for patient-wise monitoring of screening, enrolment, treatment, and control; cohort based monitoring and effective utilization of

Country	Themes			
	Access to essential medicines and devices	**Team-based care and task sharing**	**Promote continuity of services through patient centred services**	**Information systems to monitor patient and programmatic outcomes**
	essential diagnostics through Free Diagnostics Initiative and PPP models	responsibilities for health care workers at the facilities		dashboard indicators at all levels (District, State and National level)
Maldives	Morbidity/ mixed based forecasting, single combination pills are registered to be used in the country, streamlined procurement and supply chain; stock-out monitoring system at PHCs.	Health workers trained for accurate blood pressure and blood glucose measurement with provision of regular refresher training at PHC, a system of supportive supervision in place to build the capacity of PHC teams		Integrated HTN/DM clinics, systems for tracking defaulters, regular adherence counselling, existence of patient support groups /peer groups, protocol-based referral and back referral
Nepal	Morbidity/ mixed based forecasting; advance single pill combination policy; strengthen supply chain and stock out monitoring system across 3 tiers of government; automate BP measuring devices across all PHCC	Clinical mentorship		Strengthen DIHS -2 for longitudinal patient monitoring; digital dashboard for performance monitoring
Sri Lanka	Correct estimation of protocol-based drugs; make available glucose and cholesterol strips, introduced drugs in blister pack; introduce insulin single pill combinations; supply of automated, professional and clinically validated BP; introduce framework procurement agreements; ensure annual drug estimation; plasma glucose and HbA1C available through lab networks			
Thailand		Community based interventions (example- setting based approach i.e., school, healthy workplace); Health stations		Develop a NCD Registry system to consolidate nationwide data on NCD patients, using ID card–based unique identification to avoid duplicate counts, covering treatment and

Country	Themes			
	Access to essential medicines and devices	**Team-based care and task sharing**	**Promote continuity of services through patient centred services**	**Information systems to monitor patient and programmatic outcomes**
		(telemedicine) and Community Health Volunteers roles at PHC		risk factors, and integrating data from public and private sectors including Social Security Office
Timor-Leste	To develop a NCD forecasting tool, pilot test it for HTN and DM medicine requirement in few municipalities. To procure plasma glucose measurement equipment, HbA1c at CHC level, digital BP apparatus and glucometer machines and strips for all health facilities.	Build capacity for accurate blood pressure and blood glucose measurement; provide refresher training; to utilize the existing supportive supervision checklist to build the capacity of CHC teams during the visits.	Conduct same-day laboratory testing at CHC (fasting plasma glucose/HbA1C, total cholesterol, urine ketones, and serum creatinine); use job aids for patient counselling to ensure medication adherence; strengthen referral and back-referral mechanisms	Scale-up NCD e-tracker to all municipalities to longitudinally track defaulters, monitor the DM and HTN control rates. Voter id is used as a unique id to avoid duplication; initiate online NCD information systems in secondary and tertiary care levels; use digital dashboards at all levels to visualize and monitor performance on hypertension and diabetes coverage and control via TLHIS.

Priority actions on cancer control

Country	Priority action	Support requested
Bangladesh	Cancer situation assessment and Registry	QA Framework HR capacity building for registry
Bhutan	Quality assurance in cervical screening Sustaining cancer registration	Coordinate imPACT review mission Technical support for Registry
India	Day care chemotherapy center	Guidelines and case studies
Maldives	Cervical screening Palliative care clinical guidelines	Guidelines and capacity building Guidelines and HR capacity building
Nepal	Cervical screening with HPV DNA	Capacity building
Sri Lanka	Oral cancer screening Survivorship care	Developing tracking mechanism Capacity building
Thailand	Linking HBCR with PBCR Cervical screening follow-up capacity	
Timor-Leste	National Cancer Plan, Cancer Registry, Histopathology services	Technical support, technological and logistic support, capacity building

Priority actions on Tobacco control

Country	Key challenges	Way forward/ Priority actions
Bangladesh	• Delay in GYTS/ GATS due to funding gaps • Limited cessation services and access to NRT	• Allocate domestic funds for surveys • Operationalize Quitline • Include NRT in essential medicines • Integrate cessation services with other national programs
Bhutan	• Rising EENNDS/ENDS/ HTPs use • Illicit trade • Limited cessation support and access to NRT • Weak intersectoral coordination	• Strengthen BFDA capacity • Establish Tobacco Control Office • Mobilize resources for enforcement • Community engagement • Scale-up community cessation programs
DPR Korea	• No regular prevalence surveys • Absence of pictorial health warnings on cigarette package	• Introduce pictorial warnings (short-term) • Conduct regular national smoking surveys integrated with STEPS (mid-term)
India	• High prevalence of smokeless tobacco • Aggressive surrogate advertising/ brand stretching • Tobacco industry interference • Variability in enforcement of tobacco control laws/policies • Loopholes in laws	• Continue to enforce smoke-free laws • Raise tobacco taxes • TAPS enforcement • IEC-Pan India • Generate data specific to smokeless tobacco use and strengthen policies • Enhance monitoring system • Healthy Campus Initiative
Maldives	• Enforcement of generational ban • Monitoring new law • Sustaining compliance	• Fully implement Generational Tobacco Ban • Strengthen enforcement and compliance mechanisms • Expand National Quitline • Implement Tobacco Control masterplan
Myanmar	• High smokeless tobacco use • Weak enforcement of old law • Affordable and accessible tobacco products • Limited cessation access • Tobacco industry interference	• Develop and enforce new Tobacco Control and Taxation Laws • Expand smoke-free spaces • Integrate cessation counselling into PHC • Secure funding and strengthen coordination
Nepal	• Tobacco industry interference and weak enforcement • Complex tax structure • High smokeless tobacco burden and unregulated industry	• Increase tobacco taxes • Enforce FCTC-compliant laws ban e-cigarettes • Conduct mass media campaigns and expand cessation services
Sri Lanka	• High smokeless tobacco use • Taxation gaps • Industry interference and illicit trade • Limited surveillance	• Increase tobacco excise tax share to ≥75% • Integrate cessation in PHC • Regulate smokeless tobacco and strengthen surveillance and enforcement • Mitigate industry interference

Country	Key challenges	Way forward/ Priority actions
Thailand	• Rise of e-cigarettes/nicotine pouches among youth • Weak online enforcement and industry interference	• Conduct NRT cost-benefit analysis • Integrate tobacco cessation services with other national programs • Strengthen TAPs enforcement • Review TC law for full FCTC alignment
Timor-Leste	• Illicit tobacco trade • Limited enforcement and weak coordination • Funding shortages • NRT stockouts	• Develop national strategy and enforcement tools • Strengthen NRT supply • Ratify illicit trade protocol • Increase HR and budget for implementation

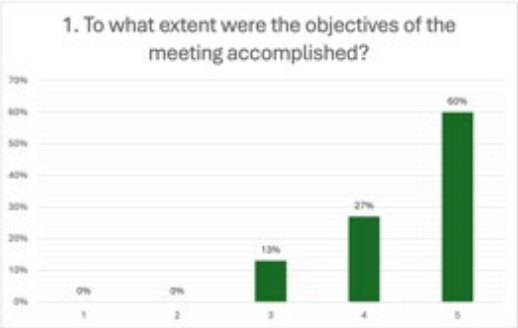
Healthy diet, salt reduction, TFA control- Challenges and way forward

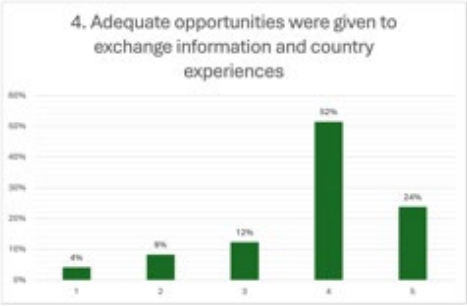
Country	Key challenges	Way forward/ Priority actions
Bangladesh	• Lack of comprehensive data on sodium intake • Absence of salt/sodium labelling on some packaged foods • Contradiction between BSTI standards & WHO benchmarks • Limited lab capacity for trans-fat testing • Industry reluctance to adopt FoPL	• Finalize sodium benchmarks and prioritize food categories for reformulation • Finalization of FoPL design • Advocate for industry compliance with reformulation and labelling • Develop lab capacity for TFA testing and monitoring • Implement roadmap via multisectoral committee
Bhutan	• Low awareness of harmful effects of TFA and PHO • Limited capacity to test nutritional content (salt, sugar, TFA) • Competing priorities and conflicting mandates across agencies • Cultural/traditional dietary practices • Limited affordability and availability of fruits/vegetables	• Include TFA and salt reduction actions in MSAP 2025–2030 • Conduct food market surveys and regulate imports • Build capacity for nutritional testing • Develop awareness materials and food-label literacy tools • Strengthen multisectoral collaboration Implement healthy canteen/catering guidelines
India	• Need more localized BCC/IEC campaigns on salt reduction • Limited capacity of CHOs, ANMs, FLWs for dietary counselling • Need more public awareness on avoiding trans fats • Need stronger interdepartmental coordination	• Continue “Aaj Se Namak Kam” + Eat Right India campaigns • Build capacity of CHOs, ANMs through training modules • Maintain nationwide TFA surveillance through FSSAI labs • Strengthen NMAP multisectoral coordination • Continue awareness campaigns for trans-fat-free foods
Maldives	• Financial constraints for surveys and situation analyses • Limited national technical capacity • Industry influence on regulation	• Advocate for government budget for nutrition policies • Build national technical capacity

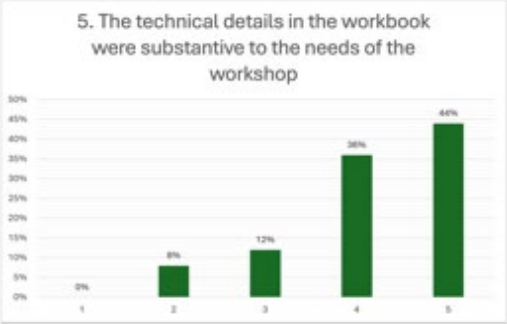
Country	Key challenges	Way forward/ Priority actions
		• Conduct desk reviews, market assessments, and stakeholder consultations
Nepal	• Limited country capacity, data gaps, small cottage industries producing packaged foods, industry interference • Diverse food habits; food insecurity; affordability issues, increasing availability of processed foods	• Technical support for stepwise implementation • Develop national standards for healthy diets • Seek WHO technical support for standards and guidelines
Sri Lanka	• Enforcement of salt reduction, monitoring systems, multisectoral engagement	• Implement population salt reduction roadmap • Strengthen monitoring and surveillance systems • Enhance public education on salt reduction • Improve multi-agency coordination
Thailand	Sodium taxation: • Food industry resistance (cost, technology limitations) • Lack of funds and technology among small food businesses Public food procurement: • Scaling up implementation across sectors remains challenging	• Develop sodium benchmarks for food categories • Promote the adoption of lower-sodium salts with potassium and other substitutes • Increase public knowledge on sodium consumption • Implement public food procurement guidelines in schools, hospitals, army, etc. Pilot and scale nationwide by 2026 • Monitor the implementation by National Sodium Reduction Committee and report the progress 2 times/year.
Timor-Leste	• Weak enforcement and monitoring systems • Multisectoral collaboration and accountability • Limited lab capacity for TFA testing	• Finalize and endorse national food labelling regulation • Ban sale of unhealthy foods in schools • Sustain BCC campaigns targeting communities & schools • Legal revisions on draft regulation to ban imports of foods with high TFA • Build lab capacity and enforcement systems • Seek WHO technical support and cross-country learning

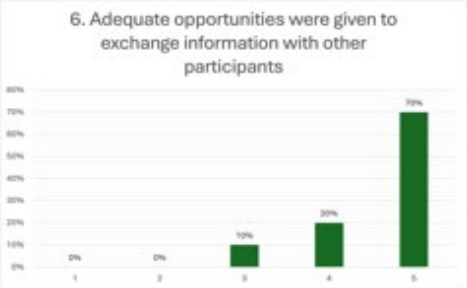
Annex 6

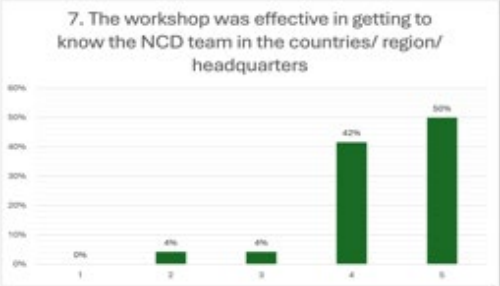
Participant's feedback

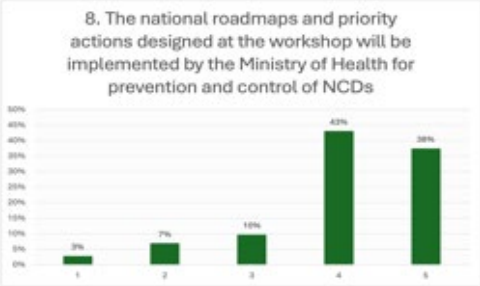


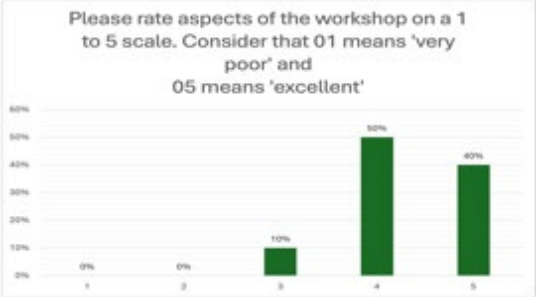












Annex 7

Resource materials presented at the Regional Workshop

- Day 1 Presentations:
  https://drive.google.com/drive/folders/1AlhBYRLbx0fyZA7QEImM9xRbnq7vNPLO

- Day 2 Presentations:
  https://drive.google.com/drive/folders/1FpF811wrSNmYF6w1MBmK3YLw8ZlYY0xG

- Day 3 Presentations:
  https://drive.google.com/drive/folders/1uuZ5FHvwmslt5MQWj5mi0y7EHNvVYIzj

Annex 8

Additional resources

- SEAHEARTS Monitoring Framework. https://iris.who.int/server/api/core/bitstreams/f3c14f02-7af7-480e-b348-7c63fbd4abe0/content
- Noncommunicable disease facility-based monitoring guidance. https://www.who.int/publications/i/item/9789240057067
- WHO HEARTS Technical Package. https://www.who.int/publications/i/item/9789240001367
- WHO Package of Essential Noncommunicable (PEN) disease interventions. https://iris.who.int/server/api/core/bitstreams/b9f09202-a320-4c07-ba2c-afe0d1186339/content
- Global Diabetes Compact. https://www.who.int/initiatives/the-who-global-diabetes-compact/
- Noncommunicable Diseases Data Portal. https://ncdportal.org/
- The Global Health Observatory. https://www.who.int/data/gho
- WHO South-East Asia Regional NCD Roadmap. https://apps.searo.who.int/whoroad/homev2
- SEARO NCD dashboard. https://whosearo.viewzenlabs.in/home
- Assessing national capacity for the prevention and control of noncommunicable diseases: report of the 2023 country capacity survey in the WHO South-East Asia Region. https://iris.who.int/server/api/core/bitstreams/8100c2b6-b336-4488-9212-553273499e18/content
- Noncommunicable diseases progress monitor 2025. https://iris.who.int/server/api/core/bitstreams/7a228681-a190-4c29-b2a8-9d4255dc49d1/content
- Second Regional Consultation (virtual) on SEAHEARTS Initiatives: Achievements and Lessons learnt: https://www.who.int/southeastasia/publications/i/item/SEA-NCD-115
- Third Regional Consultation on SEAHEARTS: Extended and Expanded Milestones 2030: https://www.who.int/southeastasia/publications/i/item/sea-ncd-116
- WHO report on the global tobacco epidemic, 2025: warning about the dangers of tobacco. https://www.who.int/publications/i/item/9789240112063
- Two decades of WHO FCTC implementation in South-East Asia Region. https://www.who.int/publications/i/item/9789290229568
- Electronic cigarettes: Call to Action. https://www.who.int/publications/m/item/electronic-cigarettes---call-to-action
- E course: Monitoring methodologies for diet related regulatory interventions to support a healthy food environment https://whoacaemy.org/coursewares/course-v1:WHOAcademy-Hosted+H0081EN+2025_Q1
- What are healthy diets? Joint statement by the Food and Agriculture Organization of the United Nations and the World Health Organization https://iris.who.int/bitstream/handle/10665/379324/9789240101876-eng.pdf
- REPLACE: WHO package for the elimination of industrially produced trans-fat https://www.who.int/publications/i/item/9789241511347
- Trans fat elimination. https://resolvetosavelives.org/wp-content/uploads/2023/05/7-Step-Trans-Fat-Elimination-Guide-1.pdf

- Trans fat elimination: Maximizing lives saved through policy implementation and enforcement https://resolvetosavelives.org/resources/trans-elimination-and-policy-enforcement/
- WHO South-East Asia Regional Strategy for comprehensive cancer prevention and management (2024-2030). https://www.who.int/publications/i/item/9789290211709
- The Cervical Cancer Elimination Initiative: https://www.who.int/initiatives/cervical-cancer-elimination-initiative
- The Global Initiative for Childhood Cancer: https://www.who.int/initiatives/the-global-initiative-for-childhood-cancer
- The Global Breast Cancer Elimination Initiative: https://www.who.int/initiatives/global-breast-cancer-initiative
- imPACT reviews and Rays of hope initiative. https://www.iaea.org/services/review-missions/impact-reviews

World Health Organization

South-East Asia Region

SEA-NCD-117